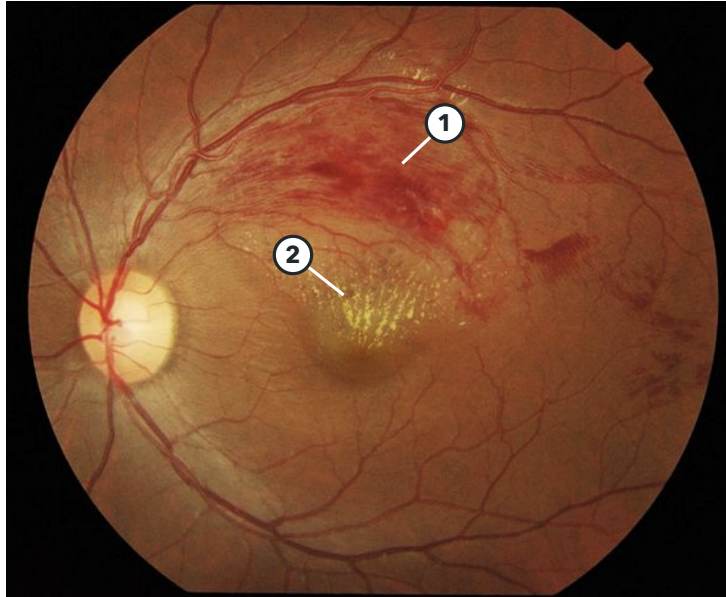


RVO: sector treatment and safety checks

Clinical M3-M5 | Focused Type B notes

The branch territory sets the laser target

BRVO: follow the drainage sector



1: haemorrhagic sector. 2: macular exudates. OCT assesses current oedema; angiography can map nonperfusion. The photo does not itself prove active retinal new vessels. Supplied M3 Anki, media 222.

Match the complication to treatment

Complication	Treatment direction
Vision-limiting macular oedema	Discuss intravitreal therapy and monitoring.
Retinal neovascularisation in BRVO	Sector photocoagulation to ischaemic retina.
No neovascularisation	Do not infer a routine laser requirement from haemorrhage alone.

Document what changed

Separate acuity/OCT response from new-vessel surveillance. Sector disease can affect the macula, and treatment for central fluid does not replace monitoring of the peripheral territory.

References RCOphth: RVO guideline, 2022 | RCOphth: RVO executive summary

Before choosing an injection or a blood test

Treatment safety

Choice	Check before and after
Anti-VEGF	Ability to attend repeated treatment and review.
Dexamethasone implant	IOP/glaucoma, previous steroid response, lens status and cataract risk.
Any intravitreal procedure	Consent, infection precautions and urgent reporting of postoperative deterioration.

Systemic RVO work-up

- 1 Check BP, glucose and vascular-risk history; obtain basic investigations according to the pathway.
- 2 Use unusual age, bilateral/recurrent disease or systemic findings to guide further evaluation.
- 3 Do not order routine inherited-thrombophilia testing or anticoagulation solely for an ordinary isolated RVO.

A longer-lasting implant still needs review

Fewer injection visits do not remove pressure and cataract monitoring. Steroid risk is especially relevant in an eye with glaucoma or a prior substantial steroid response.